

التهاب الزائدة الدودية الحاد — Acute Appendicitis

TEST FIXTURE — synthetic structural document for automated tests. Not a medical reference.

Learning objectives

- Describe the typical migration of pain in acute appendicitis.
- List the investigations used when the diagnosis is uncertain.

Clinical presentation

Pain usually begins in the periumbilical region and later migrates to the right iliac fossa (McBurney's point).

يبدأ الألم عادةً حول السرة ثم ينتقل إلى الحفرة الحرقفية اليمنى عند نقطة McBurney.

Anorexia and nausea are common; vomiting usually follows the onset of pain.

A white cell count above $11 \times 10^9/L$ supports the diagnosis, but a normal count does NOT exclude it.

Investigations — الفحوصات

Ultrasound is the first-line imaging test in children and in pregnant women.

CT abdomen is preferred in adults when the diagnosis remains uncertain after clinical assessment.

A pregnancy test (β -hCG) is required in women of reproductive age.

Differential diagnosis

The differential diagnosis includes mesenteric adenitis, ectopic pregnancy and right ureteric colic.

يجب استبعاد الحمل خارج الرحم عند النساء في سن الإنجاب.

Table 1: Alvarado score components

Alvarado score (MANTRELS) — مكونات المقياس		
Feature	Points	Unit / threshold
Migration of pain to RIF	1	—
Anorexia	1	—
Nausea / vomiting	1	—
Tenderness in right iliac fossa	2	—
Rebound tenderness	1	—
Elevated temperature	1	$\geq 37.3\text{ }^{\circ}\text{C}$
Leukocytosis	2	$> 10 \times 10^9/\text{L}$
Shift to the left	1	neutrophils $> 75\%$

Total score ranges from 0 to 10.

Management pathway

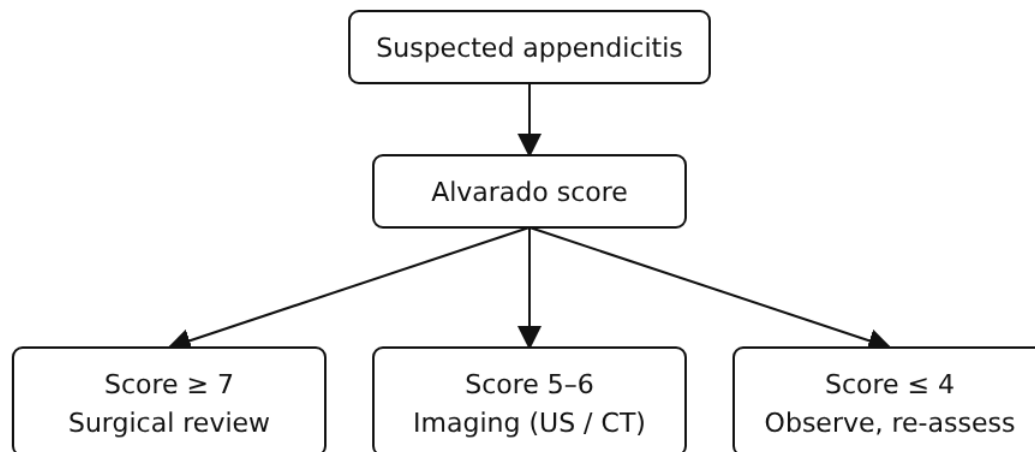


Figure 1: Management pathway by Alvarado score (synthetic diagram).

As shown in Figure 1, the score directs the next step: surgical review, imaging, or observation.